CRISIS PLAYBOOK · NO. 06

WEEK OF 14 DECEMBER 2026

- T+00:00 Incident declared
- T+00:02 Alert sent · push, SMS, voice
- T+01:30 Escalated · no response
- T+04:10 Team acknowledged



HEALTHCARE

Major incident, minute one.

Ransomware on the pathology system, a chiller failure in a heatwave, a surge in winter. How to declare, call staff in, switch to paper and keep patients safe when the systems go down.

T+00:00 · T+00:05 · T+00:15 · T+01:00 · T+04:00

INSIDE

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A practical playbook for NHS trusts and independent healthcare providers: how to declare, mobilise staff, run downtime procedures and keep patients safe in the first hour of a business continuity, critical or major incident, including a cyber attack on clinical systems.

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FOREWORD

In healthcare, an IT incident becomes a patient safety incident within the hour.

On 3 June 2024 a ransomware attack on Synnovis, the pathology provider for major hospitals in south east London, stopped normal blood testing. Guy's and St Thomas' and King's College Hospital declared a critical incident the next morning.[1] NHS England later reported that 10,152 acute outpatient appointments and 1,710 elective procedures had been postponed at the two trusts.[2] Because blood could not be matched in the usual way, NHS Blood and Transplant asked O type donors to book urgently.[3]

In June 2025 King's College Hospital confirmed that a long wait for a blood test result, caused by the attack, was one of the factors that contributed to a patient's death.[4] In April 2026, almost two years on, one mental health trust in the area was still working in business continuity for pathology.[5] And in the summer of 2026 trusts declared critical incidents when chillers failed in a heatwave and when IT systems went down.[6,7]

This playbook is for the people who carry the bleep when that happens: accountable emergency officers, EPRR and business continuity leads, on-call managers and directors, digital, nursing and communications teams, in NHS trusts and in independent providers that deliver NHS-funded care. It covers the first hour, the downtime that follows and the plan to be ready before the next one. The companion intelligence report sets out the evidence in full.

The Crises Control team
Crises Control

INTRODUCTION

Three things decide how the first hour goes.

Every trust has an incident response plan and a business continuity plan. The ones that cope best on the day have rehearsed three simple things until they are habits.

Declare early, in the right words

Use the NHS England definitions for business continuity, critical and major incidents, and the standard message formats, so partners know exactly what you mean.[8]

Know who is coming in

Reach on-call staff, ward leaders and off-duty teams in minutes, and know who has acknowledged, who is on the way and who has not answered.

Make downtime safe

Paper requests, manual result reporting, safe blood and clear handovers, practised before the systems fail, so patient care does not depend on the network.

The network can go down. Patient care cannot.

A cyber attack, a power cut or a failed cooling plant all end in the same place: clinicians working without the systems they rely on, and patients who need to be told what is happening.

PART 01

Declare and decide.

Agree the words, the thresholds and who can say them, before the day you need them.

01 Three declarations, exact words

02 Why declaring early matters

03 A healthcare severity ladder

DEFINITIONS

Three declarations, in NHS England's words

From the NHS EPRR Framework, version 3, July 2022. Use them exactly, and always say who declared it.[8]

DECLARATION	DEFINITION	WHAT IT MEANS IN PRACTICE
Business continuity incident	"an event or occurrence that disrupts, or might disrupt, an organisation's normal service delivery, to below acceptable pre-defined levels. This would require special arrangements to be put in place until services can return to an acceptable level."	Your business continuity plan is running. Examples in the framework include a surge in demand, a utilities breakdown and significant equipment failure.
Critical incident	"any localised incident where the level of disruption results in an organisation temporarily or permanently losing its ability to deliver critical services; or where patients and staff may be at risk of harm."	Principally an internal escalation. You need support from partners across the system to restore normal operating functions.
Major incident	Defined by the Cabinet Office and JESIP as "an event or situation with a range of serious consequences that require special arrangements to be implemented by one or more emergency responder agency."	Usually multi-agency. There are no universal triggers; the decision is made in a local, operational context.
Format	"Critical Incident declared by (organisation)". For major incidents: standby, declared, cancelled or stand down.	Tell your ICB, which reports to NHS England through regional on-call.[8]

DECLARE AND DECIDE

Why the first decision is the hardest, and the most important

> **Declaring is how you get the right people moving. Waiting for certainty puts you two hours behind the wards.**

Nobody wants to declare too soon. A critical incident is a public signal, it pulls in partners and it can feel like an admission. So the first hour often goes on fixing the problem quietly, while wards improvise and the on-call director hears about it second hand.

The NHS framework is clear that a critical incident is principally an internal escalation response to increased system pressures or disruption to services that will affect the ability to deliver safe patient care.[8] It exists to bring in help. When East Kent Hospitals declared one over IT problems in July 2026, it said the declaration let it keep patients safe by drawing on support from colleagues across the health system.[7]

Declaring is also how you get the right people moving. An on-call manager who declares a business continuity incident at 08:15 can start the downtime procedures, call in the pathology and digital leads and warn the emergency department. One who waits for certainty at 10:00 is two hours behind the wards.

So set the triggers in advance. Loss of a clinical system that supports results, prescribing or patient flow for more than a set time. Any suspected cyber attack on a clinical system. Loss of power, water, cooling or medical gases to a clinical area. A surge that takes the organisation to its highest operational pressure level under the OPEL framework.[9] Write them down, give named roles the authority to declare and tell them they will never be criticised for declaring early and standing down.

Use the agreed words. NHS England asks for the format “Critical Incident declared by (organisation)”, and the four standard major incident messages: standby, declared, cancelled and stand down.[8] Share facts with partners in METHANE or SBAR form, which the framework sets out, including an example for a cyber incident.[8]

Then tell your own staff, in the same words, at the same time. A declaration that reaches the ICB before it reaches the wards has gone the wrong way round.

SEVERITY

A healthcare severity ladder for the first hour

An internal scale for alerting and staffing. It sits alongside the formal EPRR declarations, not instead of them. If in doubt, grade up and step down later.

SEV 1

Major or critical, trust wide

TRIGGERS

Loss of pathology, EPR or prescribing across sites; suspected ransomware; mass casualty major incident declared by your trust or the ambulance service; loss of power or water to clinical areas.

WHO IS ALERTED, HOW

Executive on call, AEO, incident team, all clinical leads, digital, comms; ICB on call informed at once.

`Push, SMS and voice · ack in 5 min`

SEV 2

Critical in one service or site

TRIGGERS

One site's emergency department, theatres or diagnostics unable to work safely; critical system down beyond the agreed time; OPEL 4 with patient safety risk.

WHO IS ALERTED, HOW

Director and manager on call, service and site leads, digital, comms.

`Push and SMS · ack in 10 min`

SEV 3

Business continuity incident

TRIGGERS

Planned downtime overrunning; single system or equipment failure with a working fallback; staffing gap in one area; supplier outage.

WHO IS ALERTED, HOW

Manager on call, service leads, EPRR lead.

`Push and email · ack in 30 min`

SEV 4

Watch and prepare

TRIGGERS

Supplier warning, heat or cold alert, cyber advisory affecting your systems, rising pressure.

WHO IS ALERTED, HOW

EPRR lead, digital, site team.

`Email and app · no ack needed`

PART 02

Mobilise.

Reach the right people, know who is coming, and say the same thing to everyone.

01 The first hour, minute by minute

02 Call-in and acknowledgement

03 Messages ready to send

04 Three scenarios

THE FIRST HOUR

Monday, 08:10: the lab goes dark

An illustrative acute trust. Treat it as a cyber attack until proved otherwise.

CLOCK	ACTION	OWNER
T+00:00	**Laboratory staff report the system is down** Lab manager calls the site manager and digital service desk. No results are going out.	LAB MANAGER
T+00:05	**On-call manager grades it SEV 1** Business continuity incident declared. Executive on call and AEO alerted. Digital isolates systems.	MANAGER ON CALL
T+00:10	**Call-in alert sent** Incident team, pathology, transfusion, ED, theatres, critical care, comms. Reply with arrival time.	SITE MANAGER
T+00:15	**Downtime procedures start** Paper requests and phoned urgent results; transfusion to its downtime plan. Clinical leads review today's planned care.	PATHOLOGY LEAD
T+00:20	**Cyber reporting route** Digital lead calls the NHS England cyber security operations centre, which is monitored 24/7.[10] NIS operators prepare a DSPT report within 72 hours.[11]	CHIEF DIGITAL OFFICER
T+00:30	**Critical incident declared** “Critical Incident declared by (trust)”. ICB on call told; testing mutual aid requested.	EXECUTIVE ON CALL
T+01:00	**Messages out, first meeting held** Holding lines to staff, today's patients and ED. Facts in METHANE form; decisions logged.	INCIDENT DIRECTOR

CALL-IN

Five rules for calling staff in

Most trusts have a cascade. Few know, in real time, who has answered.

01 Alert by role, not by name

Lists are built around roles and rotas: the on-call transfusion scientist, the theatre coordinator, the matron on duty. People change; roles do not.

02 Ask for an answer, not a read receipt

Every call-in asks a question: available now, available later, not available. Silence after the set time escalates to a deputy automatically.

03 Use more than one channel

Push, SMS and voice. If the incident is a cyber attack, assume trust email and Teams may be down or untrusted.

04 Protect people off duty

Ask for what you need and no more. Track hours so the people who come in at 08:30 are not still there at midnight. Plan the second shift in the first hour.

05 Stand down in the same way

Release staff with a message as clear as the one that called them in, and tell them when the hot debrief will be.

REMEMBER

If you cannot see who has acknowledged within ten minutes, you do not yet know whether you have a response team.

MESSAGES

Six messages ready to send

Short, specific and honest. Fill the brackets, do not speculate on cause, and always give the time of the next update.

SMS · ON-CALL STAFF 134 chars

> [Trust] critical incident: lab systems down. Incident team to [room] now. Reply 1 coming, 2 later, 3 not available. Next update 09:00.

Send by role. Escalate no reply after 10 minutes.

PUSH · ALL CLINICAL STAFF

> Pathology systems unavailable. Use paper request forms and phone urgent results via [extension]. Transfusion is on downtime procedures. Do not use [system] until told. Update at 09:30.

Pin it in the app; repeat at each handover.

SMS · PATIENTS BOOKED TODAY 133 chars

> [Trust]: we have an IT problem affecting blood tests. Please attend as planned unless we contact you. Info: [web page]. Do not reply.

Approved by clinical leads first.

WEB AND PHONE · PUBLIC HOLDING LINE

> We have declared a critical incident because of an IT problem affecting pathology. Emergency care is open. Please use NHS 111, your GP or a pharmacy for non-urgent needs. We will update this page at [time].

Same words on the switchboard message.

EMAIL · ICB AND PARTNERS

> Critical Incident declared by [trust] at [time]. METHANE summary attached. Requests: urgent testing mutual aid, ambulance awareness. Next sitrep [time].

From the incident log, not from memory.

SMS · COMMUNITY AND LONE STAFF 118 chars

> [Trust]: IT incident. Visits go ahead. Use paper notes, call [number] for results. Check in at 12:00 and on finishing.

Ask for a reply to confirm they are safe.

IN PRACTICE

Three providers, three first hours

ACUTE TRUST, THREE SITES

Ransomware at a pathology partner

THE GAP

The lab is run by a partner; nobody at the trust was on its incident call list.

THE CHECK

A joint exercise with the partner, timed from first report to first paper result.

THE FIX

Joint call-in list and shared incident log with the partner; downtime forms printed on every ward.

WATCH OUT

Transfusion needs its own downtime plan; agree O type use with the blood bank.

COMMUNITY TRUST, 1,200 FIELD STAFF

EPR outage during a cold snap

THE GAP

Visit lists live only in the electronic record; staff were already on the road.

THE CHECK

Every lone worker confirmed safe by the end of the shift.

THE FIX

Daily offline visit lists, a check-in rule for lone workers and SMS updates that need no app login.

WATCH OUT

About one in seven NHS staff reported physical violence from patients or the public in the 2025 staff survey.[12]

INDEPENDENT PROVIDER, NHS CONTRACT

Cooling failure in a heatwave

THE GAP

Theatres and the server room relied on the same chiller plant.

THE CHECK

Temperature triggers written into the severity ladder.

THE FIX

A business continuity incident declared and the ICB told; lists moved to a partner site.

WATCH OUT

The NHS Standard Contract requires providers of NHS-funded services to comply with EPRR guidance.[8]

PART 03

Keep patients safe.

Downtime, partners, patients and staff, from the first hour to the last debrief.

01 Downtime that works

02 Working with system partners

03 Winter, heat and long incidents

04 Your plan

KEEP PATIENTS SAFE

Downtime procedures: the plan you hope never to use

> **Can a ward request an urgent blood test, get the result and act on it safely with no computers at all?**

Most clinical areas have a downtime folder. The question is whether anyone has opened it this year. The test is simple: can a ward request an urgent blood test, receive the result and act on it safely with no computers at all? If the answer depends on one person who knows how, it is not a procedure.

Synnovis showed how long downtime can last. NHS England reported that services in south east London were largely restored by the end of 2024, but in early 2026 one trust estimated that entering 161,560 pathology reports into patient records had been delayed, and it had recorded 122 patient safety incidents involving incorrect, unavailable or delayed results.[5] Recovery has its own risks: results back-entered weeks later, duplicate records and patients lost to follow-up.

Blood is the sharpest edge. When hospitals cannot match blood in the usual way they rely on O type blood, which is safe for all patients. That is why NHS Blood and Transplant appealed for O negative and O positive donors within a week of the attack.[3] Every transfusion laboratory needs a downtime plan agreed with clinicians, including who authorises O type use.

Good downtime procedures are short, printed and practised. They name the forms, the phone numbers, the runners and the order of priority: emergency department, critical care, maternity, theatres, then the rest. They say how results will be checked and entered when systems return. And they are tested in exercises with the people who will use them, not just the people who wrote them.

Treat the cyber attack as different from the power cut in one way: you may not be able to trust the systems even when they come back. Keep digital and clinical leads in the same incident meeting. The 2017 WannaCry review found that local staff shared information by personal mobile phones and WhatsApp when email was lost.[13] Decide your out-of-band channel now, before staff choose one for you.

SYSTEM PARTNERS

Who needs to hear from you, and when

A declared incident is rarely yours alone. Agree these routes in advance and test them in exercises.

ICB and NHS England

Tell your ICB on call when you declare. The ICB reports to NHS England through regional on-call, and coordinates mutual aid.[8]

Ambulance service

If you need diverts or redirection, say so early. The framework lists ambulance redirection as a reason to notify.[8]

Suppliers and partners

Pathology, imaging, EPR and facilities partners belong on your call-in lists, and you on theirs.

GPs, social care, the public

Primary care, care homes and patients need to know what still works and where to go instead.

Report cyber incidents to the NHS England cyber security operations centre straight away.[10] Operators of essential services report network and information system incidents through the DSPT within 72 hours.[11]

LONG INCIDENTS

Winter, heat and the incident that does not end

Many healthcare incidents are rising tides, not explosions. They need a different rhythm.

PRESSURE	WHAT OFTEN HAPPENS	WHAT GOOD LOOKS LIKE
Winter surge	Escalation drifts from OPEL 3 to 4 without a declared incident or extra staff.	Triggers that link OPEL levels to your severity ladder and call-in lists.[9]
Heat	Chillers and server rooms fail together; theatres and scanners stop.	Temperature triggers, cooling checks and a plan for diagnostics, as trusts learned in June 2026.[6]
Long cyber recovery	Staff run on paper for weeks with no rota change or welfare plan.	A recovery cell, shift patterns, welfare checks and a plan for back-entering results.
Lone and community staff	Visits continue with no way to raise an alarm if systems are down.	Check-in times, one-touch alarms and visit lists that work offline.
Communication fatigue	Updates stop once the crisis feels normal.	Fixed update times for staff and patients until stand down.

YOUR PLAN

This week, this month, this quarter

Owners are suggestions. Adjust them to your structure.

WHEN	ACTIONS	OWNER
This week	Write declaration triggers and who can declare. Check call-in lists by role, with tested numbers for every on-call rota.	Accountable emergency officer
This week	Confirm the out-of-band channel if email and Teams are lost. Print downtime forms for every clinical area.	Chief digital officer
This month	Agree downtime plans with pathology, transfusion, imaging and pharmacy partners, including results back-entry.	Chief medical officer
This month	Load the six message templates. Set a lone worker check-in rule for community teams.	Head of comms; chief nurse
This month	Swap incident contacts with every clinical supplier. Agree how they will alert you and how you will alert them.	Chief operating officer
This quarter	Link OPEL levels, heat and cold alerts to the severity ladder before winter peaks.	EPRR lead
This quarter	Run a no-notice call-in test and a cyber downtime exercise with a key supplier. Time first paper result.	EPRR lead
This quarter	Report results, gaps and dated actions to the board alongside the annual EPRR core standards return.	Accountable emergency officer

FOR LEADERS

Questions for your executive team

Ask them out loud. Vague answers are the finding.

READINESS

- ? Who can declare a critical incident tonight, and do they know the triggers?
- ? How long would it take to know who is coming in after a call-in?
- ? Could each ward get an urgent blood result with no computers?

PARTNERS AND PEOPLE

- ? Are our pathology, imaging and EPR suppliers on our incident call lists, and are we on theirs?
- ? How would we reach community staff if the record system and email were both down?
- ? Who looks after the people who worked the first twelve hours?

CHECKLIST

Ready for minute one?

Tick what is true today. Anything blank is your next action.

- [ ] **Declaration triggers written and owned**
  Using the EPRR definitions and standard formats.

- [ ] **Call-in lists by role, with acknowledgement**
  Tested out of hours in the last quarter.

- [ ] **Downtime procedures printed and practised**
  Including transfusion and results back-entry.

- [ ] **Out-of-band channel agreed**
  For use when email and Teams cannot be trusted.

- [ ] **Suppliers on your incident lists**
  And your team on theirs.

- [ ] **Message templates approved**
  Staff, patients, partners, public and lone workers.

- [ ] **Lone worker check-in rule**
  Works with no access to the record system.

- [ ] **Hot debrief within 48 hours of stand down**
  As the EPRR framework expects, with welfare support.

GLOSSARY

Words worth knowing

EPRR
Emergency preparedness, resilience and response: the NHS England programme and framework for incidents and emergencies.

Accountable emergency officer
The board-level executive responsible for EPRR in an NHS-funded organisation.

Business continuity incident
Disruption below acceptable levels that needs special arrangements until services recover.

Critical incident
Localised loss of ability to deliver critical services, or risk of harm to patients and staff.

Major incident
An event with serious consequences needing special arrangements by one or more responder agencies.

OPEL
Operational pressures escalation levels, from 1 to 4, used to describe pressure on services.

METHANE
A JESIP format for sharing incident information: major incident, exact location, type, hazards, access, number of casualties, emergency services.

SBAR
Situation, background, assessment, recommendation: a structured way to escalate.

Downtime procedure
How a service works safely when a clinical system is unavailable.

Mutual aid
Help from other organisations, such as testing capacity or staff, during an incident.

Hot debrief
A short review straight after an incident, within 48 hours of stand down.

DSPT
The Data Security and Protection Toolkit, used by health bodies to report security incidents.

ICB
Integrated care board: plans and coordinates NHS services across a local area, with 24/7 on-call.

Out-of-band channel
A way to communicate that does not depend on the systems that may be compromised.

WHY COORDINATION, NOT JUST NOTIFICATION

Alerting is minute one. Coordination is everything after.

When pathology goes down at 08:10, the alert is minute one. Keeping patients safe depends on who declared what, which staff came in, which downtime procedures started, what partners and patients were told, and a record for the after-action review.

01 · ALERT

Ping

Reach people on push, SMS, voice and email, and track who acknowledged.

02 · COORDINATE

Incident Manager

One workspace and log for the whole team.

03 · EXECUTE

Task Manager

Procedures become owned tasks with deadlines and escalation.

04 · SEE

Control Centre

A live picture of tasks, people, locations and blockers.

05 · PROVE

Audit Trail

A timestamped timeline, ready for the post-incident report.

A MASS NOTIFICATION TOOL	AN OPERATIONAL INCIDENT COORDINATION PLATFORM
Sends a message	Runs the response, from first alert to recovery
Knows the message was delivered	Knows who acknowledged, and escalates silence
Ends when the alert goes out	Follows the incident with tasks, owners and deadlines
Leaves the record in inboxes and chats	Keeps one timestamped timeline you can hand over
Gives leaders a list of recipients	Gives leaders a live picture of what is happening

Crises Control is an **Operational Incident Coordination Platform**. Mass alerting is where it starts, not where it ends.

PLATFORM

Where Crises Control fits in a healthcare incident

How the steps in this playbook map to Crises Control modules.

Call in on-call staff and clinical leads by role, see who has acknowledged and escalate non-responders to deputies automatically.	**Ping** Mass alerting by push, SMS, voice and email, with acknowledgement tracking and escalation on no response.
Run the incident from one workspace with one log, shared by the incident team, digital, partners and the ICB liaison.	**Incident Manager** A workspace per incident: team coordination, a single communication log and message templates.
Turn downtime procedures, transfusion steps and results back-entry into tasks with owners and deadlines.	**Task Manager** Digitised procedures: tasks assigned with owners, deadlines and escalation paths.
See which sites and services are affected, what has been done and where escalations are waiting.	**Control Centre** A live view of tasks, communications, locations and escalations.
Give community and lone staff a one-touch alarm with their location.	**SOS** One-touch mobile escalation with GPS location for frontline staff.
Keep a timestamped record of every message and decision for the hot debrief and the post-incident report.	**Audit Trail** Automatic timestamps and incident timelines, ready for the post-incident report.

SELF-ASSESSMENT

How coordinated is your response?

Score each area from 0 (not in place) to 3 (fully in place) for your healthcare organisation, then add up the total.

AREA	THE QUESTION	SCORE 0 TO 3
01 **Alert**	Can you reach the right people on more than one channel within minutes, day or night? GOOD LOOKS LIKE Clinical, on-call and support staff reached on several channels, at any hour.	0 1 2 3
02 **Acknowledge**	Do you know who has seen the alert, and does silence trigger escalation automatically? GOOD LOOKS LIKE Call-in responses tracked, with escalation when key roles stay silent.	0 1 2 3
03 **Escalate**	Are escalation paths set by role and time, not by whoever remembers who to call? GOOD LOOKS LIKE Declaration and escalation follow the EPRR framework by role.	0 1 2 3
04 **Coordinate**	Is there one shared workspace and log for the incident, not scattered calls, chats and inboxes? GOOD LOOKS LIKE One incident log shared by the incident team, clinical leads, partners and communications.	0 1 2 3
05 **Execute**	Are your procedures turned into owned tasks with deadlines that someone can track? GOOD LOOKS LIKE Downtime procedures run as owned tasks in every affected department.	0 1 2 3
06 **See**	Can leaders see tasks, people, locations and blockers live, without having to ask? GOOD LOOKS LIKE A live view of staff mobilised, services affected and actions outstanding.	0 1 2 3
07 **Prove**	Could you produce a timestamped timeline for a regulator, insurer or inquiry within a day? GOOD LOOKS LIKE A timeline that supports EPRR assurance, the ICO and the after-action review.	0 1 2 3
08 **Rehearse**	Do you exercise the whole response, not just the call tree, at least once a year? GOOD LOOKS LIKE Exercises that include a cyber attack on clinical systems, not just a major incident.	0 1 2 3

0 to 8
Alerting only
You can send messages. The response still depends on people and luck.

9 to 16
Coordinating in parts
Some steps work. Gaps appear at night, at scale or when systems fail.

17 to 24
Coordinated and provable
You can run the response and show exactly what happened.

BUYER'S GUIDE

Eight questions to ask any incident platform supplier

Many tools can send an alert. These questions separate a notification tool from a platform that coordinates the whole response.

01 **Does it stop at the message?**
Ask what the platform does after the alert is sent: tasks, log, escalation, recovery.

02 **Can it prove who acknowledged?**
Delivery is not acknowledgement. Ask how silence is detected and escalated.

03 **Does it run your procedures?**
Ask whether your plans become owned tasks with deadlines, or stay as documents.

04 **Is there one shared picture?**
Ask what leaders see live, and whether it needs someone to update a spreadsheet.

05 **Does it work when your systems do not?**
Ask how teams coordinate if email, chat or your network is down.

06 **Can frontline staff raise the alarm?**
Ask how a colleague on site escalates, with their location, in one action.

07 **What evidence does it leave behind?**
Ask for a sample timeline of the kind a regulator, insurer or inquiry would want.

08 **Can you rehearse in it?**
Ask whether you can run exercises on the same platform you would use for real.

YOUR TAILORED DEMONSTRATION

Bring your worst day. We will run it with you.

A 30-minute session built around your own healthcare scenario, not a standard product tour. You leave with a clearer view of your gaps, whether or not you go on to use Crises Control.

BRING A SCENARIO LIKE

01 A cyber attack that takes a clinical system or pathology offline.

02 A major incident that needs staff called in out of hours.

03 A winter pressure or heatwave critical incident across several sites.

WHAT THE 30 MINUTES COVERS

05 min — We agree your scenario and the people involved.

10 min — We run your first hour live: alert, acknowledgement and escalation.

10 min — We turn your procedure into tasks and show the live picture leaders would see.

05 min — We produce the timeline and talk through what it means for your plans.

Request your tailored demo

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www.crises-control.com

Request a demo

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Every clinician reached, every patient safe.

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